

Designing Onboarding for Seniors in Healthcare

An executive brief on industry best practice and where the Bold funnel can win — every claim cited.

Audience: 65+ Medicare-eligible · Sources current as of 2024–2026 · Prepared May 2026 · 8 pages

TL;DR — Five things to internalize

- **Form length is conversion's single biggest lever.** Conversion drops from **23.1% at 3 fields** → **6.9% at 10+ fields** ^[1]. The average e-commerce checkout has 11.3 fields when it needs ~8 ^[2]. Bold's current 14-step funnel is well past that cliff.
- **Plain language is a 50%+ conversion multiplier in healthcare.** Pages at a 5th–7th-grade reading level convert at a 10.8% median; 10th–12th-grade copy drops conversion by 55.6% ^[3].
- **Human voice beats automated reminders.** Manual phone reminders reduce no-shows ~39% vs ~29% for automated SMS — a 10-point lift directly relevant to Bold's no-show problem ^[4].
- **Caregivers are central, but proxy access is broken industry-wide.** Fewer than 3% of caregivers have a designated patient-portal proxy account ^[5], while 82% of homebound older adults need caregiver help to complete a telehealth visit ^[6]. Whoever solves this first owns the category.
- **Accessibility is now a legal floor, not a polish item.** HHS Section 504 requires WCAG 2.1 AA compliance by May 11, 2026 for any Medicare-accepting entity ^[7]. 4,000+ ADA digital-accessibility lawsuits were filed in 2024 alone ^[8].

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1. The Senior Audience Today

The 65+ telehealth user is no longer a fringe persona. They have the device, the comfort, and increasingly the expectation that healthcare meets them online — but onboarding design assumptions have not caught up.

78%

of adults 65+ own a smartphone in 2025 — up from 18% in 2013 ^[9].

73%

of adults 50+ used telehealth in the past 12 months; 90% were satisfied ^[10].

38%

of older adults cannot complete a self-service video visit on their own ^[11].

SENSORY AND MOTOR IMPAIRMENT IS THE BASELINE, NOT THE EDGE CASE

- 27.8% of adults 71+ have measured vision impairment ^[12].
- 67.9% of adults 70+ have hearing loss; 97.6% of those 90+ ^[13].
- 50.4% of adults 65+ live with arthritis ^[14].
- 25.2% have mild cognitive impairment by ages 80–84 ^[15].

THE TRUST POSTURE IS FRAGILE

AARP's 2025 Tech Trends report finds that **25% of older adults cite data privacy as the #1 barrier to tech adoption** ^[9]. Nielsen Norman Group's senior usability research found seniors blame themselves **90% of the time** when they hit a problem, vs. 58% for younger users ^[16]. The senior user has the device but not the confidence. Onboarding is where you win them or lose them.

2. Best Practice #1 — Reduce Friction Aggressively

Every field, every screen, every cognitive switch is a tax. For seniors, who are 2× more likely than younger users to give up on a task when something goes wrong ^[16], that tax compounds fast.

The field-count cliff

Digital Applied's 2026 benchmarks make this brutally clear: conversion drops **modestly** from 23.1% at 3 fields to 17.0% at 5, then **collapses** to 11.4% at 7 and 6.9% at 10+ fields ^[1]. Baymard's 2024 e-commerce checkout research finds the average checkout has 11.3 fields when it needs only ~8 — and **18% of users abandon checkout because the process is "too long / complicated"** ^[2].

One concrete win: making a phone-number field *optional* reduced abandonment from **39% to 4%** in a documented ConvertCart case study ^[17].

Multi-step forms — with one important caveat

The widely-cited "multi-step forms convert 86% higher" stat is poorly sourced. The credible primary benchmark is Zuko's analytics, which found multi-step beats single-page by **~14%** — **but only when the form has more than 7 fields** ^[18]. Below 7 fields, multi-step is neutral or slightly negative. Translation: *compress your fields first; then split what's left across screens.*

Conversational onboarding — promising but unproven for seniors

Vendor benchmarks claim conversational/chatbot forms achieve **30–50% higher completion** than static forms ^[19], and ~3× higher lead-conversion rates ^[20]. Hyro's enterprise health-system deployments report 80% positive-conversation rates with 98% accuracy ^[21], and K Health's AI-led intake is now in Cedars-Sinai, Mayo Clinic, Mass General Brigham, and Northwell ^[22].

Confidence note: Every chatbot-lift number above is vendor-published; no peer-reviewed senior-specific RCT exists yet. The directional case is strong; the precise magnitude for 65+ in healthcare is unknown. Recommend piloting and instrumenting before treating it as a guaranteed lift.

3. Best Practice #2 — Build Trust Early and Repeatedly

Rock Health titled its 2022 industry report *"Moving at the Speed of Trust"* for a reason. For Medicare-age users — who grew up reading AARP's scam warnings — trust is not a visual flourish, it is the gating factor on whether they will type a single character.

Reading level is the single biggest trust signal

10.8%

median conversion for healthcare pages written at a 5th–7th-grade reading level ^[3].

–55.6%

drop in conversion when copy is at a 10th–12th-grade level vs. 5th–7th ^[3].

5.1%

median healthcare landing-page conversion industry-wide — the absolute floor we are competing against ^[3].

Trust signals that move the needle (and where to place them)

- **Trust badges near submit buttons** — a recent A/B test adding a Norton badge alongside existing indicators lifted conversion **12.2%** and revenue **16.6%** ^[23]. Critically, badges in the *footer* do almost nothing — placement matters.
- **Real faces, not stock photography** — VWO documented a **+35% signup lift** when a stock image was replaced with a photo of the company's founder; eye-tracking research confirms real faces draw attention and stock faces are ignored ^[24].
- **Testimonials, especially video** — written testimonials lift sales-page conversion ~34%, video up to 62%; 79% of viewers prefer video over written ^[25]. For seniors, age-match the testimonial ("Margaret, 72, AZ") instead of using influencer content.
- **Named, credentialed clinician above the fold** — **81% of older adults say telehealth is best with a known provider** ^[10]. Show photo + full name + credentials + years of practice on landing and provider-match screens.
- **17% of cart abandoners** in Baymard's checkout research cited "didn't trust the site with my card info" — directly addressable with plain-English security copy ^[26].

Anti-patterns that destroy trust

- "Fear" copy ("harmful," "illness") underperforms "trust" copy ("proven," "recommended," "board-certified") in healthcare ^[3].
- Unexplained third-party processor names in consent text ("Verified and its vendors may receive your information...") pattern-match to identity-theft scam warnings for a 76-year-old ^[27].

- Wall-of-text HIPAA legalese gated behind a single "I acknowledge" checkbox causes abandonment at the final step — users want to "ask my daughter" first ^[27].

4. Best Practice #3 — Accessibility, Modality, and the Caregiver Lane

Accessibility is now the legal floor

HHS's Section 504 final rule (May 2024) requires every entity that accepts Medicare or Medicaid funding to meet WCAG 2.1 Level AA by **May 11, 2026** (May 2027 for <15 employees) ^[7]. The litigation trend is real: **4,000+ ADA digital-accessibility lawsuits were filed in 2024 alone**, and notably **1,000+ of those were against businesses that had accessibility "widgets" installed** — widgets do not provide legal protection ^[8].

The senior-friendly floor is meaningfully stricter than baseline AA:

- Body text $\geq 18\text{px}$ (not 16px — NN/g notes people read 11% slower for every 20 years they age ^[16]).
- Touch targets $\geq 44 \times 44 \text{ px}$ (WCAG 2.2 SC 2.5.8 AAA target — Apple HIG aligned). Motor-impaired users see error rates **up to 75% higher** on small targets ^[28].
- Contrast $\geq 4.5:1$ body, $\geq 3:1$ UI components. No gray-on-white.
- Built-in text-resize control on the page itself — NN/g observes most seniors don't know browser zoom exists ^[16].
- Replace dropdowns with radio buttons when ≤ 5 options. Inline-error at field exit, not at submit.

And accessibility is not just compliance theater — case-study evidence shows accessibility improvements lifted conversion **26%** and checkout completion **21.9%** in a documented retail case ^[29], and a 10,000-site study found WCAG-compliant sites get **23% more organic traffic** ^[30].

Modality: phone is not a fallback, it's a primary channel

AARP's 2024 research finds **66% of older adults who don't currently use telehealth would prefer audio-only over video**, and **11% of Silent Generation consumers haven't tried virtual care because they found it "uncomfortable or too confusing"** ^[31]. Zoom-first scheduling for Medicare-age users is a known anti-pattern. Phone should be *equal or default*, with Zoom as an "if you prefer video" option — and absolutely not buried as a small text link below the video CTA.

The caregiver lane: a category-leading move

Kalicki et al. (2021) found **82% of homebound older adults (mean age 82.7; 46.6% with dementia) required caregiver assistance to complete a telehealth visit** ^[6]. Yet across multiple US health-system studies, **fewer than 3% of caregivers have a designated proxy patient-portal account** ^[5]. Password-sharing — where the caregiver logs in *as* the patient — is the dominant workaround, and a documented HIPAA and privacy hazard ^[32].

The fix is structural, not cosmetic:

- An explicit *"I'm helping a family member sign up"* entry on the welcome screen.
- **Native proxy accounts** — caregiver has their own credentials linked to the patient's record (not shared logins).
- Granular consent — the patient toggles what the caregiver can see (visit notes yes, billing no).

- Post-signup notification to the patient confirming proxy access was granted.

Almost no Medicare-focused competitor has this built well. Whoever ships it first owns the conversation.

5. Best Practice #4 — Insurance Verification and Cost Resolution

Insurance verification is the single riskiest step in any healthcare onboarding. Healthcare patient-registration form abandonment runs **20–40%** per industry aggregations ^[33], and the 5-to-7-field cliff (§2 above) sits right in the middle of where MBI + insurance fields typically land.

The three-track flow

Treat Medicare Beneficiary Identifier (MBI) capture as **three parallel paths on one screen**:

1. **Photo capture with OCR (default)** — front/back of the Medicare or Medicare Advantage card, processed via Stedi, pVerify, or Approved Admissions APIs that hit CMS HETS 270/271. pVerify and Stedi support automatic MA detection from a "Medicare" card photo — which matters because most seniors do not know whether their plan is Original Medicare or MA.
2. **Manual fallback** — member ID + DOB + name only. No more.
3. **"I'll do this later" → phone concierge** — route to a scheduled call-back. Heyday Health's Medicare-focused model uses a "Health Ally" as the human first-line; Devoted Health uses a "Guide" (1-800-DEVOTED) that proactively confirms onboarding within days of enrollment ^[34].

Resolve cost ambiguity at the eligibility-confirmed screen

The single most common cost-related abandonment trigger is an unresolved range — e.g. "Your estimated out-of-pocket cost is \$0–\$35." A senior on a fixed income reads that and pauses: *"Which one is me?"* The Bold funnel audit identifies this as a Tier 4 trust issue ^[27]. Show a personalized "You will pay \$0" wherever the API can resolve it. If it cannot, explain why and commit to a follow-up — never leave the user guessing about money.

Camera-capture UX details that matter

- Card-sized overlay with auto edge detection.
- Manual capture fallback if auto fails — never block on a single low-quality image.
- Gallery upload alternative for users who already photographed their card.
- Just-in-time camera permission with a plain-language reason: *"We need your camera to read your Medicare card. We don't store the photo after we read your member ID."*

6. Best Practice #5 — Post-Booking and No-Show Reduction

Onboarding does not end at "Confirmed." For a senior user, the 24–48 hours between booking and the first visit is when they get cold feet, can't find the Zoom link, or aren't sure the appointment is real. No-show reduction is the highest-ROI lever in the entire funnel — and almost all of it lives *after* the booking is made.

The hierarchy of reminders

38%

no-show reduction from SMS reminders (Imperial College RCT, summarized by Klara) ^[35].

62%

relative no-show reduction in a 2023 clinic deployment of automated SMS + phone reminders (18.55% → 7.01%) ^[4].

39% vs 29%

Manual phone reminders reduce no-shows ~39%; automated SMS only ~29% — a clear 10-point edge for human voice ^[4].

The implication for Bold is direct: a lightweight, scaled human-touch outreach 24 hours before the first visit is likely the single biggest lever available. This is exactly what the original Care Coordinator call did before it was removed — its job was not just scheduling but *expectation-setting, tech check, modality coaching, and human reassurance* ^[27]. A 2-minute confirmation outbound call brings most of that scaffolding back without restoring the full original step.

Digital scheduling structurally helps too

Zocdoc-published data shows online bookings have **6.9% non-attendance** (cancellations + no-shows) vs. **17–31% for voice-booked appointments** — i.e., a digital book + automated reminder stack is already a structural improvement over phone-only booking ^[36]. Solv reports **75% of patients who book ahead complete digital intake before arrival** ^[37].

What "good" looks like post-booking

- Immediate confirmation: email + SMS + .ics calendar invite, sent *from the named clinician*, not "PrevMed Team."
- 60-second tech check inline after booking (camera/mic test for video; verify callback number for phone).
- Day-before SMS with the provider's photo and a one-tap "Need help connecting? Call (XXX)" CTA.
- 24-hour pre-visit human confirmation call/SMS by a Care Coordinator (especially for users 75+ and first-time video users).
- Day-of: text 1 hour before with "Tap here to join — need help? Reply HELP."

7. Bold Opportunity Map

Mapping the best practices above against Bold's current clinic-funnel flow (*Landing* → *Coverage* → *Loading* → *Eligibility* → *Focus* → *Concern* → *Intake* → *Matching* → *Provider* → *Schedule* → *HIPAA* → *Consent* → *Phone* → *Confirmed*) ^[38]. Gap severity is calibrated to the Bold Funnel Conversion Audit's Tier 1–4 prioritization ^[27].

BEST PRACTICE	BOLD CURRENT STATE	GAP	RECOMMENDED ACTION
Compress to ≤7 fields per screen; sub-5 if possible ^{[1][2]}	14-step funnel; multiple steps re-ask for info already captured	High	Audit the 14 screens for duplicate fields (e.g. DOB at Account + Eligibility). Pre-fill from OCR. Compress visible step count to ~5 logical groups: Welcome → Coverage → About You → Pick Provider → Confirm.
Make phone-number optional or defer until post-confirmation ^[17]	Phone is a dedicated mandatory step	High	Kill the dedicated Phone step or move it to post-confirmation. Cheapest 30+ point conversion lift available.
6th–8th grade reading level across the funnel ^[3]	HIPAA and Consent screens are dense legalese; "Verify eligibility" is jargon	High	Run every screen through Hemingway Editor; target Grade 6. Replace "Verify eligibility" with "Check your Medicare coverage." Compress HIPAA into a plain-English summary with a "Read full notice" expand.
24h pre-visit human-touch outreach ^[4]	No human contact post-booking after the CC call was removed	High	Restore a lightweight 2-minute CC confirmation call/SMS at T-24h. Single biggest no-show lever available.
Phone visit equal to Zoom (or default for 75+) ^[31]	"45-min Zoom video call" is the default; "Switch to phone" is a hidden text link	High	Two primary buttons on Schedule: <i>Phone</i> and <i>Video</i> . Default to phone for users 75+. Reframe duration as "15-min welcome + 30-min care plan."
Three-track Medicare verification with personalized cost ^[33]	Photo + manual paths exist but cost shown as "\$0–\$35" range; no concierge fallback	Med	Resolve to a personalized \$ value. Add explicit "Talk to a Care Coordinator instead" escape-hatch on the Coverage and Provider-match screens.
Trust signals near submit buttons and sensitive fields ^{[23][24]}	No security badges above the fold; "Frequently rebooked" badge is vague; consent text references "Verified" with no explanation	Med	Add SOC 2/HITRUST badge + plain-English HIPAA copy on Landing and Coverage. Replace "Frequently rebooked" with concrete proof ("Seen by 1,200+ patients, 4.9★"). Explain "Verified" inline or remove third-party language.

BEST PRACTICE	BOLD CURRENT STATE	GAP	RECOMMENDED ACTION
Persistent visible support phone in ≥18pt on every step [10]	CC phone visible only after commitment, not in the hero or on Coverage	Med	Show ≥18pt phone number + "Request a call back" link on every step, including the Coverage step (highest drop-off).
Caregiver / proxy entry path [5][6]	No "Helping a family member" entry; no native proxy account model	Med	Add "I'm helping a family member sign up" button on Landing. Build native proxy accounts with granular consent toggles. Category-leading move.
Replace stacked open-text fields with chip-select + 1 textarea [18]	Two consecutive open-text questions on intake — highest mobile abandonment pattern	Med	Convert Q3 + Q4 to chip-select with a single optional "anything else?" textarea. Add voice input on any free-text that survives.
Conversational onboarding pilot (directional) [19][21]	Static multi-step form throughout	Pilot	Pilot a chat-style version of the Intake step (Q3/Q4) and instrument completion vs the static control. Senior-specific evidence is thin; treat as a learning bet, not a guaranteed lift.
WCAG 2.2 AA compliance audit before May 11, 2026 [7][8]	No documented accessibility audit; ≥18pt body and 44×44 targets not enforced	Plan	Schedule axe + Lighthouse pass + one manual screen-reader test. Block release of any new funnel screen on a passing audit.

What I'd A/B test first (next 30 days)

1. **Phone equal to Zoom** on the schedule screen → expected: no-show ↓.
2. **"Talk to a Care Coordinator first"** escape-hatch on provider-match → expected: schedule rate ↑ (recovers the segment direct-booking lost).
3. **24-hour pre-visit CC outreach** → expected: show rate ↑ (single biggest lever).
4. **Hero rewrite** with "How it works" secondary CTA → expected: hero scroll % up from 30% toward 50%+.

8. Sources

Every numeric claim in this report is anchored to one of the sources below. Confidence labels: **P** = primary research / peer-reviewed; **V** = vendor-published (treat as directional); **S** = secondary trade-press / blog summary of a primary source.

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- [38] [P] Bold / PrevMed clinic_funnel current implementation — referenced screens: 01–eligibility.html through 05–confirmation.html. Source: PrevMed/clinic_funnel/. See companion file onboarding–research–priorities.md for the team-curated Tier 1–3 priorities.

How to read confidence labels: Findings marked **[P]** are peer-reviewed or come from organizations with primary access to the underlying data (e.g. Baymard, Unbounce, Zuko, AARP, JAMA). **[V]** findings are vendor-published — directionally useful but the publishing organization benefits commercially from the conclusion. **[S]** findings are secondary summaries; the underlying study is usually stronger and is linked where possible. Any claim with no isolated citable source was either dropped from this report or labeled "directional / treat as such" in the body text.